

1. SBP <90 mmHg — hypotension

WHAT YOU SEE

BP cuff reading '<90 mmHg' with a red X on the Nitrate Danger Board.

WHAT IT ENCODES

Nitrates are contraindicated when SBP <90 mmHg (or >30 mmHg below baseline) because venodilation drops preload and worsens hypotension. Give cautiously and avoid if hemodynamically unstable.

BOARD TRAP

Nitrates relieve pain but never improve mortality — don't withhold a beta-blocker or reperfusion to chase pain control with nitrates.

2. RV infarction — crossed out

WHAT YOU SEE

Heart icon with red X.

WHAT IT ENCODES

Nitrates are dangerous in right-ventricular infarction (inferior STEMI with RV involvement): the RV is preload-dependent, so venodilation causes profound hypotension. Treat RV infarct with fluids, not nitrates.

BOARD TRAP

Inferior STEMI + hypotension + clear lungs + raised JVP = RV infarct — give IV fluids and AVOID nitrates.

3. PDE5 inhibitor within 24h — crossed out

WHAT YOU SEE

'24H' pill with red X.

WHAT IT ENCODES

Nitrates are contraindicated within 24 hours of sildenafil/vardenafil (48h for tadalafil) due to synergistic vasodilation and life-threatening hypotension. Always ask about ED drugs before nitrates.

BOARD TRAP

PDE5-inhibitor use is an absolute nitrate contraindication — the classic fatal hypotension pairing; tadalafil needs a full 48h.

4. 48H PDE5 (tadalafil) — crossed out

WHAT YOU SEE

'48H' pill with red X.

WHAT IT ENCODES

Tadalafil's long half-life extends the nitrate-avoidance window to 48 hours. The shorter-acting sildenafil/vardenafil require 24h. Co-administration causes refractory hypotension.

BOARD TRAP

Tadalafil = 48h window, not 24h — mixing up the interval is a board trap.

5. CHEW — sublingual nitro

WHAT YOU SEE

Referee labeled CHEW near the NITRO conveyor.

WHAT IT ENCODES

Sublingual nitroglycerin (or chewed) is first-line for anginal chest pain; escalate to IV nitroglycerin for ongoing ischemia, hypertension, or pulmonary edema. It is symptomatic, not prognostic.

BOARD TRAP

Persistent pain despite max nitrates means proceed to reperfusion — don't keep titrating nitrates as the 'treatment.'

6. Beta-blocker Truth Wall — real contraindications

WHAT YOU SEE

Red panel 'REAL CONTRAINDICATIONS' on the Beta Blocker Truth Wall.

WHAT IT ENCODES

Early IV/oral beta-blockers reduce reinfarction and arrhythmia but are withheld when signs of cardiogenic shock or heart-failure decompensation are present (COMMIT trial caution).

BOARD TRAP

Don't give beta-blockers to a STEMI patient in/near cardiogenic shock — they precipitate decompensation (Killip III-IV).

7. PR interval / heart block — contraindication

WHAT YOU SEE

'PR interval' and 'HEART BLOCK' icons in red.

WHAT IT ENCODES

Beta-blockers are contraindicated with high-grade AV block (2nd/3rd degree) or marked first-degree block, since they further slow AV conduction. Common with inferior MI vagal tone.

BOARD TRAP

PR prolongation / AV block is a true beta-blocker contraindication — slowing conduction can produce complete heart block.

8. HR <50 bpm / SBP <90 — contraindication

WHAT YOU SEE

'<50 BPM' and '<90 BP' red tiles.

WHAT IT ENCODES

Bradycardia (HR <50) and hypotension (SBP <90) are contraindications to beta-blockade — negative chronotropy/inotropy would worsen perfusion. Stabilize hemodynamics first.

BOARD TRAP

Bradycardic or hypotensive STEMI = hold the beta-blocker; these vitals are real contraindications, not 'fake' ones.

9. Cardiogenic shock / Killip III-IV — contraindication

WHAT YOU SEE

'CARDIOGENIC SHOCK — KILLIP III-IV' red tile.

WHAT IT ENCODES

Killip class III (pulmonary edema) and IV (cardiogenic shock) are absolute contraindications to early beta-blockade. COMMIT showed early IV metoprolol increased shock in such patients.

BOARD TRAP

The COMMIT lesson: early beta-blockers in decompensated/shock patients increase cardiogenic shock — avoid until stable.

10. Asthma / bronchospasm — contraindication

WHAT YOU SEE

'ASTHMA / BRONCHOSPASM' red tile.

WHAT IT ENCODES

Active bronchospasm/severe asthma is a contraindication to non-selective beta-blockade; use cardioselective agents cautiously if needed. Severe reactive airway disease is the concern.

BOARD TRAP

Stable COPD is NOT an absolute contraindication — only active bronchospasm/severe asthma; cardioselective beta-blockers are often tolerated.

11. FAKE contraindications bin

WHAT YOU SEE

Trash can labeled 'FAKE CONTRAINDICATIONS' with discarded tags.

WHAT IT ENCODES

Diabetes, stable COPD, peripheral vascular disease, and stable HFrEF are NOT contraindications to beta-blockers — they are myths. Beta-blockers improve outcomes in these conditions.

BOARD TRAP

'Diabetes/COPD/PVD' as reasons to withhold a beta-blocker are distractors — these are fake contraindications.

12. IV beta-blocker — selective use

WHAT YOU SEE

'IV BETA-BLOCKER' panel on the green truth wall.

WHAT IT ENCODES

IV beta-blockers are reserved for ongoing ischemia/hypertension/tachyarrhythmia in hemodynamically stable patients; otherwise start oral within 24h once stable. Routine early IV use is not recommended.

BOARD TRAP

Routine IV beta-blocker for every STEMI is wrong (COMMIT harm) — give oral within 24h in stable patients, IV only for specific indications.
